Lab Report from LabCorp

Specimen Number	Patient ID	Control Number	Account Number
10922914730	110672923	5285520	04332945

Patient Last Name	Patient First Name	Patient Middle Name
BIRKNER	NICOLE	

Patient SS#	Patient Phone	Total Volume
	650-799-6867	

Date of Birth	Sex	Fasting
1999-08-04	F	N

Patient Address

2321 SCOTT ST APT 6
SAN FRANCISCO
CA, 94115-1786

Date/Time Collected	Date Entered	Date/Time Reported
2023-04-19 06:33:00 PDT	2023-04-19 21:00:00 PDT	2023-04-24 06:36:00 PDT

Account Address

One Medical Group
2410 California Street San Francisco CA 94115

Additional Information

Physician Name	NPI #	Physician ID
KONG, M	1306296348	

Tests Ordered

Lipid Panel (303756); Iron and TIBC (001321); Hemoglobin A1c (001453); Estradiol (004515); FSH (004309); Thyroxine (T4) Free, Direct (001974); Luteinizing Hormone(LH) (004283); Prolactin (004465); TSH (004259); Vitamin D, 25-Hydroxy (081950); Testosterone, Total, LC/MS (070001); Thyroid Antibodies (006684); Ferritin (004598); Vitamin B12 (001503); Progesterone (004317); Cardiovascular Report (910385); PDF Report (PDFReport1)

TESTS	RESULTS	FLAG	UNITS	REFERENCE INTERVAL	LAB
Lipid Panel					
Cholesterol, Total	172		mg/dL	100-199	01
Triglycerides	93		mg/dL	0-149	01
HDL Cholesterol	78		mg/dL	>39	01
VLDL Cholesterol Cal	17		mg/dL	5-40	01
LDL Chol Calc (NIH)	77		mg/dL	0-99	01
Comment:	Canceled				01
Iron and TIBC					
Iron Bind.Cap.(TIBC)	313		ug/dL	250-450	01
UIBC	192		ug/dL	131-425	01
Iron	121		ug/dL	27-159	01
Iron Saturation	39		%	15-55	01
Hemoglobin A1c					
Hemoglobin A1c	5.1		%	4.8-5.6	01

TESTS	RESULTS	FLAG	UNITS	REFERENCE INTERVAL	LAB
Prediabetes: 5.7 - 6.4 Diabetes: >6.4 Glycemic control for adults with diabetes: <7.0					
Estradiol					
Estradiol	129.0		pg/mL		01
Adult Female: Follicular phase 12.5 - 166.0 Ovulation phase 85.8 - 498.0 Luteal phase 43.8 - 211.0 Postmenopausal <6.0 - 54.7 Pregnancy 1st trimester 215.0 - >4300.0 Roche ECLIA methodology					
FSH					
FSH	3.2		mIU/mL		01
Adult Female: Follicular phase 3.5 - 12.5 Ovulation phase 4.7 - 21.5 Luteal phase 1.7 - 7.7 Postmenopausal 25.8 - 134.8					
Thyroxine (T4) Free, Direct					
T4,Free(Direct)	1.32		ng/dL	0.82-1.77	01
Luteinizing Hormone(LH)					
LH	4.9		mIU/mL		01
Adult Female: Follicular phase 2.4 - 12.6 Ovulation phase 14.0 - 95.6 Luteal phase 1.0 - 11.4 Postmenopausal 7.7 - 58.5					
Prolactin					
Prolactin	8.8		ng/mL	4.8-23.3	01
TSH					
TSH	2.800		uIU/mL	0.450-4.500	01
Vitamin D, 25-Hydroxy					
Vitamin D, 25-Hydroxy	41.4		ng/mL	30.0-100.0	01

TESTS	RESULTS	FLAG	UNITS	REFERENCE INTERVAL	LAB
Vitamin D deficiency has been defined by the Institute of Medicine and an Endocrine Society practice guideline as a level of serum 25-OH vitamin D less than 20 ng/mL (1,2). The Endocrine Society went on to further define vitamin D insufficiency as a level between 21 and 29 ng/mL (2). 1. IOM (Institute of Medicine). 2010. Dietary reference intakes for calcium and D. Washington DC: The National Academies Press. 2. Holick MF, Binkley NC, Bischoff-Ferrari HA, et al. Evaluation, treatment, and prevention of vitamin D deficiency: an Endocrine Society clinical practice guideline. JCEM. 2011 Jul; 96(7):1911-30.					
Testosterone, Total, LC/MS					
Testosterone, Total, LC/MS	39		ng/dL		02
This test was developed and its performance characteristics determined by Labcorp. It has not been cleared or approved by the Food and Drug Administration. Reference Range: Adult Females Premenopausal 10 - 55 Postmenopausal 7 - 40					
Thyroid Antibodies					
Thyroid Peroxidase (TPO) Ab	12		IU/mL	0-34	01
Thyroglobulin Antibody	<1.0		IU/mL	0.0-0.9	01
Thyroglobulin Antibody measured by Beckman Coulter Methodology					
Ferritin					
Ferritin	**176**	**High**	**ng/mL**	**15-150**	**01**
Vitamin B12					
Vitamin B12	1071		pg/mL	232-1245	01
Progesterone					
Progesterone	1.0		ng/mL		01
Follicular phase 0.1 - 0.9 Luteal phase 1.8 - 23.9 Ovulation phase 0.1 - 12.0 Pregnant First trimester 11.0 - 44.3 Second trimester 25.4 - 83.3 Third trimester 58.7 - 214.0 Postmenopausal 0.0 - 0.1					
Cardiovascular Report					
Interpretation	Note				03

TESTS	RESULTS	FLAG	UNITS	REFERENCE INTERVAL	LAB
Supplemental report is available.					
PDF	.				03
PDF Report					
PDF Report1	LCLS^Image^PDF^Base64^EMBEDDED DOCUMENT				03

01	01	Labcorp San Diego 13112 Evening Creek Dr So Ste 200, San Diego, CA 92128-4108	Dir: JennyGallowayMD
02	02	Esoterix Inc 4301 Lost Hills Road, Calabasas Hills, CA 91301-5358	Dir: BrianPoirierMD
03	03	Litholink Corporation 150 Spring Lake Dr Ste A, Itasca, IL 60143-2091	Dir: PawanVohraPhD

For inquiries, the physician may contact Labcorp at 800-222-7566 Lab: 858-668-3700

Accessions: 10922914730

DISCLAIMER: *These assessments and treatment suggestions are provided as a convenience in support of the physician-patient relationship and are not intended to replace the physician's clinical judgment. They are derived from national guidelines in addition to other evidence and expert opinion. The clinician should consider this information within the context of clinical opinion and the individual patient.*
SEE GUIDANCE FOR CARDIOVASCULAR REPORT: Grundy SM et al. 2018 Multisociety guideline on the management of blood cholesterol: a report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines. J Am Coll Cardiol 2019; 73: e285-350; Contois et al. Clin Chem 2009; 55(3):407-419; Brunzell et al. Diabetes Care 2008; 31(4):811-82.

Note: *Please refer to your LabCorp Report for all results as well as any test-specific and specimen-specific comments.*

Cardiovascular Report

Patient Assessment

Current available clinical information suggests the patient's risk is at least LOW. If the patient has two or more major risk factors, the risk category is intermediate. If the patient has CHD or a CHD risk equivalent, the risk category is high. If patient does not have CHD or a CHD risk equivalent, consider use of the Pooled Cohort Equations to estimate 10-year CVD risk, as individuals with greater than 7.5% risk may warrant more intensive therapy. The calculator can be found at: http://tools.cardiosource.org/ASCVD-Risk-Estimator/

Insulin resistance, obesity, excessive alcohol use, smoking, nephrotic syndrome, liver disease, and certain medications can cause secondary dyslipidemia. Consider evaluation if clinically indicated.

Therapeutic lifestyle changes are always valuable to achieve optimal blood lipid status (diet, exercise, weight management).

Lipid Management

Select one patient risk category based upon medical history and clinical judgment. Additional risk factors such as personal or family history of premature CHD, smoking, and hypertension modify a patient's goals of therapy. In CVD prevention, the intensity of therapy should be adjusted to the level of patient risk. MODERATE intensity statin therapy generally results in an average LDL-C reduction of 30% to less than 50% from the untreated baseline. Examples include (daily doses): atorvastatin 10-20 mg, rosuvastatin 5-10 mg, simvastatin 20-40 mg, pravastatin 40-80 mg, lovastatin 40 mg. HIGH intensity statin therapy generally results in an average LDL-C reduction of 50% or more from the untreated baseline. Examples include (daily doses): atorvastatin 40-80 mg and rosuvastatin 20 mg.

▽ = PATIENT'S RESULT

ANALYTE / RESULT	Patient Risk Category (select one)		
	LOW	**INTERMEDIATE**	**HIGH**
LDL-C **77 mg/dL**	▽ 160	▽ 100 130	▽ 70 100
non-HDL **94 mg/dL**	▽ 190	▽ 130 160	▽ 100 130
Lipid Assessment	LDL-C is optimal, 77 mg/dL. Non-HDL Cholesterol is optimal, 94 mg/dL.	LDL-C is optimal, 77 mg/dL. Non-HDL Cholesterol is optimal, 94 mg/dL.	LDL-C is normal, 77 mg/dL. Non-HDL Cholesterol is optimal, 94 mg/dL.
Treatment Suggestions	Considerations for use of statin therapy include family history of premature atherosclerotic disease, elevated coronary artery calcium score, ankle-brachial index < 0.9, elevated CRP, or elevated lifetime CVD risk.	Consider measurement of LDL particle number or Apo B to adjudicate need for further LDL lowering therapy. Factors that may influence statin use include family history of premature atherosclerotic disease, elevated coronary artery calcium score, ankle-brachial index < 0.9, elevated CRP, or elevated lifetime CVD risk. If statin cannot be tolerated or increased, alternatives include use of an intestinal agent (ezetimibe or bile acid sequestrant) or niacin.	If at least a 50% LDL reduction from baseline has not been achieved, begin or increase statin. Consider measurement of LDL particle number or Apo B to adjudicate need for further LDL lowering therapy. If statin cannot be tolerated or increased, alternatives include use of an intestinal agent (ezetimibe or bile acid sequestrant) or niacin.

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Patient Results Summary

Cholesterol comes in different forms and has varying effects on your heart health. Some cholesterol is “good” and not known to cause disease, this is HDL. The rest of cholesterol causes disease by clogging your arteries, this is non-HDL. LDL cholesterol is the largest component of the non-HDL cholesterol. Lowering your levels of “bad” cholesterol will lower your risk for disease.

- **LDL cholesterol (LDL-C)** is the largest component of the non-HDL cholesterol (“bad” cholesterol).
- **non-HDL** is composed of many different types of cholesterol (not just LDL-C) and high levels cause disease.

The level to which your LDL must be lowered depends on the risk for developing heart disease or having a heart attack. The higher your risk for heart disease, the lower your LDL goal.

Contributing Risk Factors For Heart Disease

☐ Heart and/or vascular disease	☐ Cigarette (tobacco) smoking
☐ High blood pressure	☐ Low HDL (men less than 40 mg/dL, women less than 50 mg/dL)
☐ Diabetes	☐ Family history of early onset heart disease
☐ Chronic kidney disease	☐ Man over 45 years or woman over 55 years
☐ Obesity	☐ Familial Hypercholesterolemia

Your Heart Disease Risk Category

Selected by your physician based upon your risk factors and clinical judgement.

Test / Your Results	☐ Low	☐ Intermediate	☐ High
LDL-C **77 mg/dL**	▽ 160	▽ 100 130	▽ 70 100
non-HDL **94 mg/dL**	▽ 190	▽ 130 160	▽ 100 130

▽ = Your Result: Left (Green) = Optimal, Center = Acceptable, Right (Red) = High Risk

Your Care Plan (as selected by your physician)

☐ Eat less trans fats and saturated fats, red meat, and sugary foods/drinks	☐ Control any other medical conditions: such as diabetes, high blood pressure
☐ Eat more vegetables, fruits, whole grains, low-fat dairy products, poultry, fish, and nuts	☐ Visit your doctor as scheduled and obtain all follow-up tests/treatments recommended
☐ Exercise	☐ Take all of your medications your doctor(s) have prescribed
☐ Lose weight	☐

Disclaimer: You should discuss this information with your physician. Labcorp does not have a doctor-patient relationship with you, nor does it have access to a complete medical history or physical examination conducted by a physician that would be necessary for a complete diagnosis and comprehensive treatment plan. Neither you nor your physician should rely solely on this guidance. Bolded result descriptions in “Comments” consider either the reference range or target range for the test result. Reference range refers to the Labcorp reference interval. Target range refers to the guideline-suggested goal. REFERENCES: National Kidney Foundation Kidney Disease Outcomes Quality Initiative (KDOQI) at www.kidney.org and Kidney Disease Improve Global Outcomes (KDIGO) at http://kdigo.org. Adapted from: https://www.niddk.nih.gov/-/media/Files/Health-Information/Health-Professionals/Kidney-Disease/Your_Kidney_Test_Results_EN.pdf

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